6.17 Acute Pancreatitis

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "Severe pain in the upper abdomen going to my back." • Gives the pain 9/10 **Objective** • Lipase raised • Guarding • Vomiting	**ACTUAL** **Acute pain** related to pancreatic inflammation as evidenced by severe epigastric pain radiating to the back.	Pain 3/10 or less within 24 hours.	• Give opioid analgesia regularly as prescribed. • Sit him up leaning forward, or let him lie with his knees bent. • At first, nothing by mouth as ordered; start feeding when it is ordered. • If the vomiting does not stop, pass an NG tube as ordered.	Pain controlled: **goal met**.
Subjective • "I am very thirsty." **Objective** • HR 118/min, BP 100/60 mmHg • Urine output 20 mL/hour	**ACTUAL** **Deficient fluid volume** related to fluid shift into tissues and vomiting as evidenced by tachycardia, hypotension and low urine output.	Within 12 hours, urine output at least 0.5 mL/kg/hour.	• Give IV fluids as prescribed. • Record urine output every hour. • Watch calcium (look for tetany) and potassium. • Vital signs every hour.	Output adequate: **goal met**.
Subjective • *No specific complaint* **Objective** • Pancreas inflamed • Glucose 190 mg/dL	**RISK** **Risk for unstable blood glucose level** as evidenced by risk factors: pancreatic injury.	Blood glucose stays on target.	• Check blood glucose every 4–6 hours. • Give insulin as ordered.	On target: **goal met**.